

# “It makes me feel part of the society”: Return-to-work decisions of SSDI beneficiaries

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## Abstract.

**BACKGROUND:** The SGA Model Demonstration included a qualitative follow-along study of participants’ return-to-work decision-making.

**OBJECTIVE:** The authors describe the findings, including motivations of SSDI beneficiaries to seek employment and vocational rehabilitation (VR) services, and the social, familial and personal considerations in returning to work.

**METHODS:** This study used a longitudinal qualitative approach, involving a series of in-depth interviews with SSDI beneficiaries who sought services from state VR agencies in Minnesota and Kentucky.

**RESULTS:** Motivations of SSDI beneficiaries to seek employment extend beyond economic need. Fear and confusion associated with balancing *financial risks and rewards of returning to work* influenced whether participants felt ready to do so. Yet while financial considerations are important, they are often embedded in or superseded by motivations related to *family engagement, roles, and responsibilities; activity and social engagement; and the personal meanings of work*. Financial and benefits counseling provided through the SGA Model was widely praised by participants, and the informed decision-making process facilitated by the coordinated team approach allowed many to envision pathways that had not previously seemed viable.

**CONCLUSIONS:** Decisions to return to work are governed by multifaceted processes. VR services that are responsive to these complexities can empower individuals to make informed decisions.

**Keywords:** People with disabilities, employment of people with disabilities, social security beneficiaries, qualitative research, vocational rehabilitation

## 1. Introduction

Identifying and attending to barriers and facilitators of obtaining employment is critical in the creation of sound policies and programs that address the needs of people with disabilities who want to work. Understanding what drives people to seek employment in the first place is also important. Considerable attention has been paid in research and practice to identifying barriers - and, to a lesser extent, facilitators - to

employment among people with disabilities (Hernandez et al., 2007; Milfort et al., 2015; Netto et al., 2016; Russinova et al., 2011), but findings vary depending on whose perspective is studied. Policy analysts and service providers have identified systemic and clinical barriers (Milfort et al., 2015; Wittenburg et al., 2013), whereas studies based on the perspectives of workers or jobseekers with disabilities highlight personal and social factors, in addition to the impact of rehabilitation and other services (Andersen et al., 2012; Netto et al., 2016). These discrepancies suggest that hearing from people with disabilities themselves reveals important data unavailable from other sources and is therefore essential to understanding what influences the success of the employment process.

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While there is some research on what factors encourage or discourage people with disabilities to return to work (Hebert et al., 2006; Mirza, 2012; Trujillo, 2010; Tschopp et al., 2011), these studies have primarily focused on individual economic factors (Ben-Shalom & Mamun, 2012, 2015; Nazarov, 2016) or other pragmatic issues, such as functioning and the accessibility of workplaces (Larson et al., 2007; Wang et al., 2014). A study of people on long-term sick leave found a link between strong motivations to return to work and success in obtaining employment (Carlsson et al., 2019). Hence there is value in stepping back to understand what drives individuals with disabilities to seek work in the first place. Some research has focused on the social and personal benefits of working, including improved mental health outcomes (Alegria et al., 2017; Becker et al., 2007; Dunn et al., 2008; Larson et al., 2007), but it is unclear whether anticipation of these benefits motivate individuals to return to work. Missing from the literature are the less tangible personal, social, and familial dynamics influencing return-to-work decisions. Understanding these factors requires in-depth qualitative research that prioritizes the narratives of people with disabilities themselves.

The return-to-work decisions of Social Security Disability Insurance (SSDI) disabled worker beneficiaries are of particular interest because these individuals have substantial prior work histories but have low employment rates (Ben-Shalom & Mamun, 2015). SSDI for disabled workers is a federal program that makes monthly payments to eligible beneficiaries based on their previous wages. Beneficiaries are subject to earnings limitations, above which they are considered to be engaging in Substantial Gainful Activity (SGA). After a trial work period, continued earnings above the SGA amount eventually bring an end to SSDI payments. Despite the risk of losing benefits, some SSDI beneficiaries decide to return to part-time or full-time employment. Financial needs and goals, as well as changes in functioning, are part of this decision-making process but may not tell the entire story. Previous studies reveal a large gap between those SSDI beneficiaries who want to work and those who actually obtain work (Liu & Stapleton, 2011; Livermore, 2009). High unemployment rates persist for people with disabilities, whose average earnings are far lower than those without disabilities (Lauer & Houtenville, 2017). In 2016, working-age adults with disabilities were twice as likely to live in poverty than those without disabilities (27% vs. 12%) (Lauer & Houtenville, 2017). SSDI benefits do

not keep many people out of poverty; in 2015, 29% of SSDI-only beneficiaries lived in households below the poverty line (Social Security Administration, 2018). In addition to economic impacts, unemployment has negative impacts on social, psychological, and physical well-being (McKee-Ryan et al., 2005).

This manuscript shares findings from a qualitative follow-along study embedded in a demonstration that tested VR service innovations, using a randomized control trial (RCT) design (Kehn & Honeycutt, 2020; Martin & Sevak, 2020). The demonstration involved developing a model to serve SSDI-only customers, and the implementation of the model by VR agencies in two states, Minnesota and Kentucky. The model was designed to meet the particular employment service needs of SSDI beneficiaries and included common elements in both states, including rapid engagement, a faster pace of services, and a coordinated team approach, which involved VR counselors, financial and benefits counselors and job placement specialists working together with VR customers. Participants for the demonstration were SSDI beneficiaries who sought VR services in Minnesota and Kentucky during the period of the model demonstration (between 2015 and 2017). The RCT compared customer outcomes in offices that were randomly assigned to either implement the model (enhanced-service sites) or not (usual-service sites) (Kehn & Honeycutt, 2020; Martin & Sevak, 2020).

The goal of this qualitative follow-along study is to understand the employment process from the point of view of jobseekers with disabilities, including how and why they decide to return to work. This manuscript highlights the personal, familial, and social factors that motivate these SSDI beneficiaries to seek work, and the role of VR in making and following through on those decisions.

## 2. Methods

This study used a longitudinal qualitative approach, involving a series of interviews with 48 SSDI beneficiaries who sought services from the VR agencies in Minnesota and Kentucky as part of the SGA Model Demonstration. It was designed to follow a sample of the demonstration's 2325 participants, in order to gain an in-depth understanding of their experiences with and perspectives on returning to work. We used a flexible and dynamic approach to conducting series of in-depth interviews, which allowed the participants to share what was most important and

salient to them. Participants were at least 18 years of age, seeking services from their state VR agency during the period of the demonstration, receiving SSDI benefits as disabled workers, and able to provide informed consent. In accordance with the RCT criteria, individuals were excluded from the study if they had a legal guardian, were receiving Supplemental Security Income (SSI) benefits, or received SSDI benefits for blindness or only as a child or widow of a deceased beneficiary. Recruitment, data collection, and data analysis procedures are described below. All study procedures were reviewed and approved by the Institutional Review Board of the University of Massachusetts Boston.

### 2.1. Participants

Recruitment was structured to protect the confidentiality of both study participants and VR customers who chose not to participate. The research team compiled recruitment packets, which included an introductory letter, fact sheet, consent forms, and a return mail envelope addressed to the lead author. The packets were shipped to the two VR agencies, and VR office staff mailed the packets to a total of 825 customers, which was a subset of the RCT participants. The recruitment materials included a dedicated email address and toll-free telephone number, both monitored by members of the study team. VR customers who received the study packet could call, email, or mail the study team if they were interested in participating or learning more. Potential participants were then screened by a researcher, and, if they met study criteria and wanted to participate, they were invited to complete the informed consent process and set up a first interview. This approach helped maintain confidentiality and reduced the chances of influence by VR staff.

Of the 48 participants included in this qualitative follow-along study, 75% were in the RCT treatment group, and 25% were in the RCT control group. Table 1 displays the demographics of our sample and that of the overall RCT sample, whose data were drawn from the agencies' case management systems (Sevak, Kehn, et al., 2017; Sevak, Martin, et al., 2017). While the age of participants closely matched the overall sample, there were slight differences in race and gender; i.e., there was a slightly higher percentage of Black women in the current study than in the RCT sample. Given that Black women face disproportionate inequities in employment and

Table 1  
Characteristics of study participants and RCT sample

|                               | Qualitative<br>study<br>N = 48 | RCT<br>Sample<br>N = 2325 |
|-------------------------------|--------------------------------|---------------------------|
| Gender                        |                                |                           |
| Male                          | 40%                            | 53%                       |
| Female                        | 60%                            | 47%                       |
| Race/Ethnicity                |                                |                           |
| Black                         | 21%                            | 16%                       |
| White                         | 77%                            | 80%                       |
| Latino                        | 2%                             | 3%                        |
| Age                           |                                |                           |
| Range                         | 26–65                          | 18–65                     |
| Median                        | 51                             | 49                        |
| Education                     |                                |                           |
| Less than high school diploma | 8%                             | 8%                        |
| High school diploma           | 15%                            | 38%                       |
| Some post-secondary education | 52%                            | 31%                       |
| Bachelor's degree or higher   | 25%                            | 22%                       |

pay (Purcell et al., 2010; Rosenfeld & Kleykamp, 2012) yet have been historically underrepresented in research (Fisher & Kalbaugh, 2011; George et al., 2013; Huang & Coker, 2010), we consider minor oversampling for this particular population a benefit rather than a deficit.

As shown in Table 1, this study's participants resembled the overall RCT sample with respect to education, with some differences. Participants were equally as likely as the overall sample to have less than high school diploma and only slightly more likely to have a bachelor's degree or higher. But a much higher percentage of qualitative study participants reported having some post-secondary education than the overall sample, and a correspondingly smaller percentage reported having only a high school education. This difference could be due to the recruitment approach, which used a packet of text-heavy documents that would be more accessible to those with a higher education level. However, given that the proportions on the upper and lower ends of the education spectrum were very similar, it could also be due to a difference in how "some post-secondary education" was defined in our study and by how it was defined by VR staff entering case data.

All participants lived in the community, with a range of family and household arrangements. One quarter (25%) were married, 33% were divorced or separated, and 42% were single. Just under half (46%) lived alone; the rest had a variety of household arrangements, including living with spouses and/or children, caring for parents or others with disabilities, or sharing a home with other family members such as

Table 2  
Number of interviews conducted by state

|              | 1st<br>Interviews | 2nd<br>Interviews | 3rd<br>Interviews | Total<br>interviews |
|--------------|-------------------|-------------------|-------------------|---------------------|
| MN           | 23                | 20                | 10                | 53                  |
| KY           | 25                | 20                | 9                 | 54                  |
| <b>Total</b> | <b>48</b>         | <b>40</b>         | <b>19</b>         | <b>107</b>          |

siblings, or living with roommates. 17% had children under 18 living with them, but many others spoke about adult sons and daughters, non-custodial children, grandchildren, nieces and nephews, and friends' children, who played a significant role in their lives.

In the interviews, participants were asked an open-ended question, "What type of disability or disabilities do you have?" and were also asked to choose one or more disability categories from a provided list. Most reported disabilities in more than one category. The most common category was "health or medical," with 67% of participants indicating a disability in that category. Over half (56%) reported a mental health disability, 33% reported a mobility disability, 31% reported a brain injury, 13% reported an intellectual or developmental disability, 10% reported a hearing disability, and 6% reported a visual disability.

## 2.2. Data collection

One to three in-depth qualitative interviews were conducted with each of the 48 participants, for a total of 107 interviews, over the course of about 18 months (Table 2). The interviews were predominantly conducted over the phone, with a few conducted in person, and generally lasted 30 minutes to an hour. Interviews were individualized; interviewers began with broad questions and pursued topics as they were raised by interviewees. The content of the interviews extended beyond planned interview questions, which covered areas such as employment goals and the job search process, current and past work, educational programs, applying for and getting SSDI, vocational rehabilitation services (including specific aspects of the model being tested), and financial issues. While the interview guide did not include direct questions about participants' disabilities (apart from the demographic questions described above), most talked at some length about their disabilities in relation to the planned topics, and interviewers probed when appropriate.

Drawing on an ethnographic approach, the interviews involved asking open-ended questions, listen-

ing for and picking up "markers" that indicated topics of importance to interviewees, and probing for further stories and details (Weiss, 1994). A key goal of ethnographic interviewing is to understand the meanings of events and circumstances within the worldview of the person being interviewed, in other words, "to know what you know in the way that you know it" (Spradley, 1979). Ethnographic research has stressed the important role of narrative in understanding and interpreting the experience of illness or injury, including healing and rehabilitation, from the point of view of the person experiencing it (Kleinman, 1988; Mattingly & Garro, 2000). An ethnographic interview is a collaborative endeavor between the interviewer and interviewee and therefore may reveal information about multiple areas of the interviewee's life, their experiences of personal importance, their social worlds, and the systems with which they interact (Heyl, 2001). Conducting multiple interviews allows a research relationship to develop and for the interviewee to tell a fuller narrative.

## 2.3. Data analysis

The interviews were audio recorded, transcribed, and entered into Atlas.ti software for qualitative data analysis. Following principles of grounded theory (Charmaz, 2005, 2006) and procedures outlined by Miles et al. (2014), data collection and analysis happened concurrently, making the research an ongoing, iterative process, as the data fed the analysis and the analysis shaped further data collection. Miles et al. (2014) write that this approach helps the researcher "cycle back and forth between thinking about the existing data and generating strategies for collecting new, often better, data." Thematic coding and memoing began early, before the first round of interviews was completed, and analysis in Atlas.ti continued throughout the study. Initial codes were created both deductively (based on areas of inquiry) and inductively (emerging out of the interviews themselves). Codes were added, modified, and clarified over the course of the study. For example, in subsequent rounds of coding, as more interviews were conducted, some codes were refined into sub-codes, while others were combined under larger headings.

All coding was reviewed by the lead researcher and revised as needed, and any significant disagreements were resolved through consensus, to improve consistency and ensure that all instances of each theme were captured. On the basis of this analysis and ongoing review of interview questions and techniques, the

interview guides were revised periodically. Follow-up interview guides were developed to probe on emerging themes, as well as more sensitive topics, such as participants' financial situations, that had not been asked about directly in the first interviews. Participants were identified for follow-up interviews after reviewing the transcripts and determining that further interviews would likely continue to provide new information of use to the study.

### 3. Results

While the participants had a range of life experiences, family arrangements, socioeconomic conditions, and work histories, several themes were heard repeatedly throughout the interviews. As all had recently applied for VR services, it is unsurprising that they universally expressed a desire to work, either immediately or in the future (e.g., after completing an educational program or recovering from surgery). However, they were nearly evenly split on whether they planned to work above SGA and eventually transition off disability benefits. In interviews, 40% expressed a desire and intention to go off SSDI, either right away or at some point in the future, and 44% planned to stay on SSDI. The remaining were unclear, unsure or ambivalent. Specific goals and plans for working differed from person to person, but there were many common themes within their narratives.

The remainder of this section focuses on major themes that arose in the study regarding participants' decisions to return to work and the role VR played in facilitating their decisions or plans. Since many of the study participants were employed at some point over the course of the interviews and all had held jobs in the past, their narratives are built from their past and present experiences of working, often sustained periods of unemployment, and their goals and hopes for the future. The findings are organized into the following four themes that reflect factors in decisions about returning to work for SSDI beneficiaries: financial risks and rewards of returning to work; family engagement, roles, and responsibilities; activity and social engagement; and the personal meanings of work. The final section describes the participants' experiences of the SGA Model.

#### 3.1. *Financial risks and rewards of returning to work*

Despite the potential benefits of work and earning an income, transitioning to full-time work was often

characterized by participants as a risky endeavor, and the fear of losing benefits led to ambivalence about following through on employment plans. The process of returning to full-time work was also seen as complicated and confusing, requiring calculating the impacts on not only cash benefits from SSDI, but also other benefits, such as health care (Medicare and/or Medicaid), food (SNAP), and housing (Section 8 and public housing), for oneself and one's family. Murray\*, a single man in his fifties, expressed ambivalence about a full-time job recommended by his VR team this way:

"Having a full-time job and benefits is very tempting. And I would really like to have that kind of security again. [But] you know, the SSDI, I don't know how much it would affect it. And then I don't know if I'd make enough to have a living wage."

Participants were aware that earning any level of income would have the potential to affect means-tested benefits, and given the complexity of the various systems, predicting impacts could be difficult without careful analysis and planning.

Sabrina, a divorced woman in her fifties, described her financial situation as "poverty level" and managed to survive on her limited income by living with her mother. She had been taking part-time, temporary jobs until she had a health crisis that prevented her from working. She saw taking the leap to earning above SGA as a "drastic change" that she was not ready to make:

"The system, the way that it's set, helps a lot of people. But it gives you just enough to hurt you in some areas. Because it's like you can't rise above where you are. And if you do, you have to make a drastic change."

In this quote Sabrina highlights a common concern among many participants; her public benefits do not keep her out of poverty, but they are a more stable source of income than employment, especially given the unpredictable nature of her disabilities. Still, she wanted to eventually go off SSDI and saw part-time work as a stepping stone towards that goal.

Even though working full-time (or above SGA) can ultimately provide an even higher income – as well as health insurance and other fringe benefits – several

\* All participant names are pseudonyms. Other proper names and identifying information in quotations are replaced by generic terms in brackets to preserve confidentiality.

other participants said they preferred working part-time and maintaining SSDI benefits. By doing so, they could still increase their overall income, but with less risk. On the one hand, full-time employment had the potential for providing earnings that were greater than their current benefits. On the other hand, many feared losing their reliable SSDI income, which was often difficult to obtain. Some saw working even part-time as a risk to their status with SSDI and feared accidentally exceeding their allowed earnings and losing benefits.

While a few participants said they were “okay” or “making it,” most were struggling, describing their financial situations with phrases like “very poor” or “pretty dire.” Several were unsure month to month whether they could cover their basic household expenses, much less spend money on leisure or social activities. Priscilla, a single woman in her fifties, described her financial situation this way:

“To me, it’s not having savings and it’s not making it through the month. And food-wise, I always go every month, lack of food, because food’s always the last thing you’re going to do. . . . And I just didn’t have time to get that budgeted. And it’s that mental, emotional depression part of all that. It’s very stressful, and it’s hard to take care of the basics and want to pursue [career goals] and not have to focus on when you’re going to eat again.”

In this quote, Priscilla drew connections between her financial worries and her mental health, which made budgeting a challenge. Other participants similarly noted the time and energy it took to manage their multiple public benefits, as well as their health care needs. Without a budget, Priscilla said she left food as the “last thing” to take care of, which could mean going without. In turn, she found it difficult to pursue her career goals while worrying about her next meal. Each of these dimensions of her life was inextricably linked to the others, highlighting the need for supports that attend to this complexity.

### 3.2. *Family engagement, roles, and responsibilities*

Financial concerns were often embedded in - and sometimes superseded by - less tangible needs and goals, including fully engaging in family life or fulfilling a role in the family. Participants who were parents spoke about providing materially for their children, but also being present for them - providing care and guidance, protecting them from harm, and acting as

a role model. Patricia, a single mother in her fifties with school-age children, spoke about the challenges of balancing these roles, especially as a single parent with disabilities. In addition to SSDI, she received food stamps and a Section 8 housing voucher; she also visited the food pantry and had help from adult family members. She wanted to be employed but did not feel able to work full-time, both due to her physical limitations and because she needed time to spend with her children. Patricia did as much part-time, temporary work as she felt she could, but she and her family still struggled to make ends meet. However, while she was concerned about her family’s economic wellbeing, she was also motivated to be a role model or “good example” for her children, which in part meant self-sufficiency:

“I’m trying to be as self-sufficient as I can because I have children, and I want to be a good example. I’m divorced and in a situation with these two children. . . . I just try to work as much as I can so we can be able to make it and be able to handle it, because just on this Social Security, on my own now, I’m struggling. It’s hard.”

Another participant, Bernard, also divorced and in his fifties, spoke about what returning to work full-time and going off SSDI would mean to him, personally and as a father. His children lived with their mother, but he continued to be actively involved in their lives, which was easier to do while he remained unemployed. At the same time, like Patricia, he wanted to be a role model to his children - and for him, that meant having a job:

“Being unemployed has been very helpful in terms of me being able to do things with my kids and getting them places where they need to go, for them, which is really good. Because if I was working, that would be a lot more challenging. . . . So certainly that’s kind of a plus side to all of this, but on the other hand. . . they know their dad doesn’t work. And I’m sure on some level, that’s got to be hard for them.”

At the time of our last interview, Bernard had interviewed for a full-time job that was in his broad area of interest but paid less than he had hoped. Given the prospect of losing SSDI for himself and his children, as well as other public benefits, he and the financial counselor at VR had determined that this position would not allow him to “break even” financially. Still, the intrinsic values of working, including setting what he saw as a good example to his children, outweighed

the financial implications of losing benefits, and he had decided to take the job if it were offered to him.

Thelma, a woman in her early 60s was concerned about fulfilling her role as a grandmother, saying, “with my medical expenses mounting, by the end of the month, I don’t have money left to do anything, and I can’t spoil my grandchildren.” Another role she could not fulfill without a job was that of an equally contributing spouse; being unemployed meant her husband (also in his early 60s) had to shoulder more of the financial burden. Instead of working part-time and easing into retirement, she lamented, he had to continue working full-time. As seen in these examples, practical concerns such as food, housing, and medical expenses motivated participants to obtain employment, but these pragmatic issues were often tied to deeper desires for family engagement and to live up to the expectations of their families.

### 3.3. *Activity and social engagement*

Most participants cited a desire to be active and increase their social engagement as motivations to seek work. They saw a higher income as opening opportunities for greater freedom of movement and leisure and also saw increased activity and socialization as intrinsic benefits of working. Unemployed and underemployed participants (who made up the bulk of the sample) often described their current daily life as inactive, unproductive, isolated, and boring. Rather than “sitting at home watching TV,” they wanted to “get out of the house,” “socialize,” and be “productive.” These statements reflect several interrelated themes, including reducing isolation and depression, being physically active, and enjoying life. Several participants said they wanted the opportunity to interact with coworkers and customers, and others wanted to use their earnings to go out with friends and family in their free time. Alice, a divorced woman in her 50s who was providing home care to a single client, spoke about the importance of “socialization,” saying:

“I moved up here and my parents were gone for three months in [another state]. They just got back a couple days ago, and I really don’t have any friends—I mean I have friends, but we don’t socialize; we’ll talk on the phone. And it’s very, very lonely and isolating. And even at work, it’s just me with one patient. But it’s better than sitting here by myself and watching TV.”

Others spoke about the mental health benefits of being around other people. George, a divorced man in his 60s with mental illness and addictions, found a part-time job in a grocery store, saying, “I like to associate with the people at work - customers and employees.” Having a job meant being less isolated and more active, in a way that felt healthy and productive, as opposed to the social interactions he associated with his addictions:

“I sure as heck don’t want to go back to what I was where I was drinking and gambling and pretty much a loner, staying home. My home was my bar. That was my thing. Boy, I sure don’t want that again. So that’s a big goal.”

George observed that his new schedule provided “a little structure” and the positive social interaction he felt he needed to maintain his sobriety. Similarly, feeling “isolated and alone” was also the “biggest motivator” to return to work for Ben, a divorced man in his 50s with a neuromuscular disease and a mental health disability. After his divorce, when “the depression got really bad” and he “got into a pretty deep hole,” he started seeking work “out of desperation,” in hopes of gaining “some confidence.” By the time of our second interview, he had found a full-time job and had just ended his last month of SSDI. Even though going off SSDI caused him some anxiety, Ben decided to keep working so he could continue to “reengage with the world”:

“Basically because I needed to reengage with the world. Work is work, good days and bad. But it was better than being isolated. . . . emotionally I was not doing so well because, other than going to [sports practice] three days a week, there wasn’t a whole lot else in my life, so work has kind of filled an important gap.”

Ben missed participating in a sport as an exercise practice but still preferred working because it reduced his overall isolation. While he admitted, “I still struggle with anxiety as well as depression,” he also felt his “emotional health” had improved because of his new routine, which included getting up early every weekday, walking his dog, and going to work, and then seeing friends on weekends. He also attributed the improvement in his mental health to the independence he felt in going off SSDI: “I think breaking that sense of neediness and dependency was an important step for me.”

Many people framed the difference between employment and unemployment as wanting to “get

out of the house,” rather than “isolating here at home.” Keith, a man in his 60s, considered volunteering because that would mean “getting out more” without losing SSDI, saying that “sitting in the apartment watching TV all day is kind of ridiculous. . . because I like to do things.” Keith and a few others saw volunteering as a way to be active and to help others with less risk than paid employment. Similarly, Alice, a divorced woman in her 50s said working was “a reason to get out of the bed, get showered, and get out of the house every day” as opposed to “not doing anything, just sitting around every day watching TV.” Beyond having more money to spend on leisure or family activities, for many participants, the act of getting up and going to work itself meant participating more in the community and living a more active life.

### 3.4. *Personal meanings of work*

Embedded in many of these stories and quotes from participants is a desire to work because of the meaning it brings to one’s life. To the individuals in this study, working meant many things, including feeling productive, contributing to society, helping others, being a role model, and attaining a sense of social belonging. These meanings participants derived from work also served as motivators in their search for employment. As mentioned above, a common theme among many participants was a desire to work in order to be “productive,” and several people linked productivity to a self-concept as a worker, such as Dawn, a married woman in her 40s:

“I’ve been on disability for, gosh, it’s been, I want to say, six years. So I’ve basically been stuck in this house, or whichever house we lived in, and I’m going crazy. I used to work a lot of hours. Prior to being disabled, I worked almost 50 hours a week and made a wonderful income off of that. And I miss it. I miss it. I really just want to get back to work, meeting new people. I loved working. I don’t like being at home. I mean I can say maybe the first month or so was kind of nice to get a break, but it’s not what I wanted.”

Like Dawn, all the participants in this study had a substantial enough work history to qualify for disabled worker benefits from Social Security, so returning to work often meant returning to what they thought of as “normal” life and fulfilling their expected social roles. Nancy, also a single woman

in her 50s, who lived with her adult daughter in a small town, felt unable to work full-time but volunteered occasionally and wanted to get a part-time job in order to “get out of the house,” saying:

“I’ve worked all my life, but right now with my limitations and my health and everything, I can’t do a whole lot. But I just wanted to get out and just be able to help somebody or do something.”

Nancy expressed a common theme in the interviews - she had “worked all [her] life” and thought of herself as “the type of person who can’t just sit still.” Her self-concept was also connected to her ability to “help somebody,” “do something,” or just “keep busy” despite her limitations. Finding a job, she felt, would help her become that person again. Similarly, Clifford, a man in his 20s with a brain injury, wanted to “try to be a stand-up human-being that is respectable.” He said he simply wanted to be a “normal person,” explaining, “part of being a normal person in my eyes is someone who works.”

Bernard, the divorced man in his 50s who was quoted above about his role as a father, wanted to return to work full-time, and linked being “a lot busier” and “a lot more productive” with feeling “engaged with society” and “giving something back,” in contrast with getting “a check every month from the government” while on SSDI. One of Bernard’s motivations for working was to help others. While he searched for a full-time job, he did volunteer work and found satisfaction in listening and providing advice to others in a mutual support group. “Even though I’m not working,” he said, helping others “helps me feel productive. . . . It provides some type of psychological lift for me.” As mentioned above, a few other people who were not ready to work full-time also found volunteer work to be a less demanding and less risky option, since it would not produce income that could jeopardize their SSDI, but it would be a meaningful way to spend their time.

Many people talked about the ways that their personal identities and feelings about their lives were tied to working, and some framed employment as building their self-esteem. For example, Holly, a married woman in her 40s with adult children, had a goal of working full-time and going off SSDI, but her road was going to be much longer. VR was helping her get the education she would need to pursue a new career in a helping profession, which had been a dream of hers. The following is an exchange from her first interview:

*Interviewer:* “What do you think it would be like to work and go off disability?”

*Respondent:* “Well, for one, my self-esteem would be built because I know that I would be doing something to help other people and that I belong somewhere, that I’m needed somewhere. And the feeling to be needed, that goes a long, long way. . . . I want to feel like I belong. I want to feel like I’m part of the society. And when you’re on disability, you’re this little group over here outside the circle with the rest of the world.”

*Interviewer:* “Does going to school help you feel like you’re part of the society?”

*Respondent:* “Oh yeah, because none of them kids know that I’m on disability. I got a 4.0; I got straight ‘A’s. . . . Those students, when they find out the grades that I have, they come to me and ask me to help them. That makes me feel so good to know that them kids are looking up to me for something. It makes me feel part of the society.”

Being a role model, building her self-esteem, and being “part of the society” were paramount for Holly. For her, working and going to school carried meanings far beyond any practical or financial benefits.

Participants often associated working, and especially working enough to go off SSDI, with self-sufficiency and independence, such as Fiona, a woman in her 30s:

“It’s kind of embarrassing with peers: “What do you do for a living?” “Oh, I’m disabled.” “You’re disabled at 26?” “Yeah.” I mean, it’s just embarrassing. And so that made me lean more towards—being so young—to make a living to where I wouldn’t have to rely on [SSDI]. But then it’s just not always that simple. . . . You feel better about it if you do it yourself than somebody handing it to you. . . . I worked for it and paid in to draw it, but still that just don’t make me feel 100 percent better about it at all.”

In this interview, Fiona struggled with the meanings of disability, work, and government benefits. Even though she recognized she was entitled to SSDI benefits after paying into the program, she felt “embarrassed” with her peers about being disabled at such a young age and would “feel better” about earning money through a paycheck. This narrative of ambivalence about deserving or needing SSDI, especially in contrast with one’s identity as a worker, arose repeatedly in the interviews. As these quotes show,

the meaning of work is closely tied to the other themes discussed here - social engagement, family roles, and fears about losing stability – and was part of what motivated participants to seek assistance from VR.

### 3.5. *Experiences of the SGA Model*

The SGA Model was designed to serve the needs of SSDI beneficiaries and emphasized rapid engagement and an increase in the pace of services, and utilized a coordinated team approach (CTA) that involved a financial and benefits counselor and job placement specialist, led by the VR counselor (Sevak, Kehn, et al., 2017; Sevak, Martin, et al., 2017). While this qualitative follow-along study was not designed as an evaluation of the model or a comparison with usual VR services, interviews did involve discussions of the services participants received, and these experiences were connected to the decisions they made about returning to work. The following sections focus on the experiences of individuals receiving the SGA Model, i.e., customers of the intervention sites. Four aspects of the model will be discussed: the *coordinated team approach*, *financial and benefits counseling*, *job placement services*, and *pace of services and client engagement*.

#### 3.5.1. *Coordinated team approach*

The coordinated team approach was led by a VR counselor and included a financial and benefits counselor and job placement specialist, who were expected to collaborate to provide clients with in-depth, early and ongoing personalized assessments, counseling, and other services. For people who had participated in typical VR services in the past, the coordinated team approach tended to be the most noticeable change. When asked, most participants were able to describe what each team member helped with, and some identified their name and/or title. For example, one described their team this way: “a *counselor* and a person that would help me with different stuff like my *finances* and then another one that would help me *look for a job*, like three different people.” Notably, the model’s broad role of the financial and benefits counselor is reflected in this quote, that is, to help with “stuff like my finances” and not just benefits planning. Scheduling team meetings was a barrier mentioned by a few participants, as it was by staff (Kehn, Livermore, & Morris, 2017), but at least one noted that this was not a problem because they had access to each provider between team meetings and understood that “I could meet with any one of

those three people if I wanted to go a little bit deeper with something.” A few participants did not like having people aside from their counselor involved, either because they saw the information as redundant or because they were not yet at the right stage of the employment process. However, most saw the team approach as a helpful or neutral aspect of the service. One participant who was “intimidated” at first, noted the efficiency of having everyone in the room together, saying “it saved time”:

“When [the benefits counselor] was present, he went over the benefits, the rules, all the stuff about what you can and can’t do to maintain SSDI, basically. When [the job placement specialist] was there we talked about, “This is what I would like to do.” “Okay, there are some job openings.” She’s reaching out and finding contacts and finding out what I have to do, who I need to talk to. I’ve got to go through training that costs money. So [the VR counselor] is checking on, is there grant money that can pay for my training?”

This person concluded that “they’re really working with me as a team; they’ve been great so far.” Others reflected on how the team approach informed their goals and contributed to positive outcomes – both tangible (e.g., finding a job) and intangible (e.g., confidence and optimism).

### 3.5.2. *Financial and benefits counseling*

Financial and benefits specialists were included in the coordinated teams to provide early and ongoing education and counseling not just about SSDI and other public benefits, but also the larger picture of client finances. They participated in developing the individualized plan for employment (IPE) and discussed financial concerns and goals. They also helped clients understand the range of state and federal benefits for which they might be eligible, the implications of job earnings on these benefits and overall finances, and options for returning to work. This aspect of the model was discussed quite a bit in the interviews, and most spoke about it positively. Still, observations varied. A few participants had already done their own research and thought the information was superfluous. However, others observed that this process made a huge difference in their employment plans. As discussed in section 3.1, anxiety and fear about the financial risks of returning to work was a common barrier to returning to work. For example, for Tony, the financial and benefits counseling cleared up his confusion and made him feel less

apprehensive about taking steps towards transitioning off SSDI:

“The financial planning lady, she had a very thorough discovery of what all of my benefits are, what all my income sources are, what they could be in the future... and whether it was worth it for me... One of the biggest worries with me at the very beginning was, “If you work too much, you’re going to lose these benefits.” ... I’ve already ran the numbers, but it was a reassurance having somebody that’s in the system run the numbers and show me.”

As this quote indicates, despite having done his own analysis of his financial situation, Tony was still uneasy until the financial and benefits specialist “ran the numbers” and discussed his situation with him. About the work incentives that would allow him to transition back to work without immediately losing benefits, Tony said:

“It is a comfort. I think it’s a great thing to have in place because it actually motivates—it allows people in my situation to make a plan like this and be comfortable moving on the plan.”

This was a common reaction to the service. Paula, a married woman in her thirties, had gone to VR for help with completing her associate degree and received the SGA Model, which was more comprehensive than she anticipated. Prior to receiving these services, Paula had not understood the implications of the work incentives programs for her and her family. Through meeting with a financial and benefits counselor at VR, she learned that she could keep her SSDI benefits for several months while trying out full-time work. If she were able to do that, Paula explained, she and her husband could put some of that money into savings:

“If we can plan it just right, that would be a huge down payment [on a house] that we could save up. And that would be wonderful for us.”

Asked if she had thought this was a possibility prior to going to VR, she replied:

“Never in a million years. No. No. I thought that I would always have to be the stay-at-home, disabled wife. I didn’t think that I could ever contribute significantly to our family. I thought we’d probably always have to live in an apartment because of the cost of my care is so high. So the fact that there’s a possibility that I

could be contributing an actual equal income is amazing.”

### 3.5.3. *Job placement services*

As part of the SGA Model, customers were also provided with early and ongoing job information and support. Job placement specialists were expected to work closely with customers and other VR staff as part of the coordinated team to develop appropriate employment goals and a job search plan that was integrated into the IPE. Job placement specialists also provided labor market information to the customer and the rest of the team, and ongoing assistance with employment preparation and the job search. Participants generally appreciated the help with resumes, applications, and job listings, and a few thought the job placement specialist played the most important role in their team. Others expected a more hands-on approach and were disappointed that no one contacted employers on their behalf. For example, in our second interview, Thelma, a single woman in her early 60s, said she was “discouraged” and wanted more from her job placement specialist than links to job listings:

“And I feel that’s fine and dandy they’re sending me these links. That’s great, but I really expected them to do a little bit more. I mean, like contact the person and say, “Look, I’m going to have this person contact you,” or, “I want you to contact this person. We’re working with this person,” but nothing like that is happening.”

Some did receive that level of assistance. For Larry, a formerly incarcerated man in his fifties, having job placement services early in the process was key to his successful outcome. In our first interview, he expressed skepticism about VR services and ambivalence about getting a job, both because of his physical limitations and financial worries. When we interviewed him for a second time a few months later, Larry had gone back to VR and had a coordinated team meeting, at which he learned that he was not at risk of losing his SSDI and Medicare benefits, which made him willing to try out working. Using their knowledge of the local job market and acting immediately on his motivation to work, Larry’s team helped him get an interview that same day. He was hired and began work at once, and he still had that job five months later when we interviewed him for a third time.

Sarah, a married woman in her fifties who was looking for a full-time job that would allow her to

go off SSDI, also had a good experience with her job placement specialist:

“We actually sat down and applied for some jobs there at the rehab office. And I know that she has called a couple of places to make sure the jobs are still open. And I think that some of the places, she kind of was familiar with the people that were posting the jobs, maybe. And if she wasn’t, another placement coordinator that worked in a different area, she could call them and say, “Have you ever worked with this company and do you know exactly what they’re looking for,” and things like that.”

Sarah had not found work by our last interview, but she attributed her difficulty to the limited job market in her small town, not to her VR providers, whom she praised.

### 3.1.4. *Pace of services and client engagement*

Another aspect of the SGA Model was an acceleration in the VR process for application, eligibility determination, and IPE development. This meant moving clients into services and through the employment process more quickly. As Larry’s experience shows, a long planning process is not always necessary. Despite the increased pacing in the SGA Model, none of the participants we interviewed thought the VR process was too fast. Some were especially satisfied with how quickly the initial application and eligibility process went, which allowed them to see a counselor right away. Some who were previous VR customers noticed and appreciated the increase in pacing. However, among both the intervention group and the control group, participants most commonly perceived the process overall to be too slow, and this was their chief criticism of the services. Several noted that, once they approached VR, they had already made the decision to seek work and wanted to start the job search process immediately and obtain employment as soon as possible. While participants often gained important insights from the VR services, they were often frustrated with their prolonged unemployment and the barriers they continued to face.

## 4. *Discussion and implications for VR practice*

This study shows that decisions about returning to work for people on SSDI are complex and dynamic, based on many interlocking variables. All partic-

ipants had some desire for employment, but fear about the potential consequences of returning to work played a role in the extent to which they felt ready to take the leap to working, either part-time or full-time. Study participants were confused about the complex governmental systems with which they were entangled, and this limited their ability to set goals and move forward. Despite these and other barriers, some chose to start on a path towards full-time work. Others intended to remain on SSDI indefinitely but still found value - financial, personal, and social - in working and earning under the SGA limit. Several individuals in this study used a period of part-time work while maintaining SSDI as a trial, to determine if full-time work might be possible for them. The early inclusion of financial and benefits counseling in the VR process helped many participants feel more confident in taking these steps.

Still, while participants considered economic factors in making decisions about returning to work, their decisions were not driven entirely by financial need. Most people in the study reported financial struggles, but even those who were relatively comfortable wanted to work because of the inherent value they derived from it. This is consistent with previous studies that have highlighted the ties between employment, social engagement, and meaning-making for people with and without disabilities (Baldrige & Kulkarni, 2017; Freedman & Fesko, 1996; Leufstadius et al., 2009; Moseley, 1988; Torres Stone et al., 2016). This study illustrates that, beyond material comforts or even practical survival, people can be motivated by a number of intrinsic benefits that employment offers. For the participants in this study, it was often important to reconnect with their identities as workers, as providers, as family members, and as active members of society, sometimes reconciling those identities with that of a person with a disability, and sometimes wanting to transcend that identity.

Both the decisions about returning to work and the process of doing so are therefore complex and involve all areas of a person's life. Because these decisions are not only financial, but based on familial, social, and personal factors, a unidimensional approach to employment services for SSDI beneficiaries may not be sufficient. Participants often had multiple complex motivations to return to work, coupled with some trepidation; thus they wanted and needed clear, individualized information, actionable goals, and practical support. A coordinated team approach, such as in the SGA Model, is one way

to ensure that the employment process is seen from multiple perspectives and that the various supports needed to move forward are working in concert.

Including the job placement specialist and the benefits and financial counselor as part of a team early in the planning process has several potential advantages. For one, it helps the customer and counselor to be better informed about the labor market and the financial implications of returning to work. Additionally, from the customer's perspective, meeting with the entire team sends a message that there are multiple sources of support they can draw on. But key to the coordinated team approach is that, not only does the customer have access to each individual service, but that the providers speak to each other with the customer as a group. Meeting as a team can help ensure that all members have a mutual understanding of the customer's goals and the steps needed to achieve those goals. Ideally, team meetings encourage creative problem-solving with the customer to address their multifaceted needs and goals. For example, early job placement services provides customers like Larry (section 3.5.3), who was seeking employment right away, with the real time labor market knowledge to assist him in finding a job quickly. The job placement specialist can also help others, like Paula (section 3.5.2), plan longer term education and training for careers in which there are local opportunities.

The financial and benefits counseling was widely praised by participants, and for some, made a significant impact on their employment decisions, opening up the possibility of "trying out" full-time work without risking losing SSDI immediately. However, the impact on public benefits were often tied to other factors that participants weighed in their decisions, indicating that a purely economic lens is insufficient. For example, after receiving the financial and benefits counseling, Paula was able to envision a future that she had not previously imagined, which could include more financial stability not just for her individually, but also for her family. But beyond that, she saw herself inhabiting a new role in her family - that of an equally contributing spouse - something that hadn't previously seemed possible. For Holly (section 3.4), education was a means to attain a personally and financially rewarding career that would make her feel "part of the society." Even when economic needs or goals were described as paramount, they were inextricably tied to opportunities for social connection and personal and familial well-being, demonstrating that the employment process is embedded in the

context of individuals' complex lives: their work histories, family contexts, personal goals and dreams, and other aspects of their social worlds.

#### *4.1. Limitations and directions for future research*

While we attempted to recruit participants from a range of background and experiences, some limitations to the study sample are important to note. The consent information was conveyed both in print and over the telephone, during the screening process and at the start of each interview, but only in English. In order to ensure fully informed consent, we excluded people with legal guardians, as well as those who could not answer a series of questions to confirm they understood their rights as study participants. Other interview formats were offered, including video conference or TTY. Still, while all participants had significant disabilities, our sample did not include people with certain cognitive or speech impairments or Deaf individuals. Nor did it include people with limited English language proficiency or members of certain underrepresented ethnic and racial groups, including Asian and Pacific Islander Americans and Native Americans.

Even though we conducted interviews over the span of 18 months, conducting interviews over a longer period could offer additional insights into how former VR customers experience their employment outcomes, including how remaining unemployed, working part-time, or working full-time and transitioning off SSDI affects their lives. Further data collection on this population and sample, both quantitative and qualitative, could reveal the outcomes of decisions and how plans and goals develop over time. This would allow exploration of the entire employment process and the effects of SSDI receipt, or loss of SSDI, on long-term outcomes.

This study's scope was limited to the experiences of individual demonstration participants, but the findings show that employment decisions are not made in a vacuum. Parents, for example, may have to weigh the need for child care and their desire to provide financial stability for their families. Families are both affected by and influence choices made about working, so family members should be recognized as agents in the decision-making process, rather than just factors in individual decisions. A study that includes family and household members as active participants could further illuminate the social nature of the employment process and how it impacts others.

## **5. Conclusion**

This project used a longitudinal qualitative approach to gather the experiences of people with disabilities who were taking steps to return to work with the help of a state VR agency. When the study was first designed as part of the SGA Project, the goal was broadly to understand how these individuals were experiencing the process of returning to work. Using a flexible and adaptable approach to conducting series of in-depth interviews allowed for emphasis to be placed on topics of importance to the participants. In doing so, the study revealed the complex nature of employment decisions. Using an open-ended approach, we came to see that exploring how and why the participants were returning to work was central to understanding their experiences of employment and vocational rehabilitation. We found that how and why SSDI beneficiaries decide to work are governed by complex, multifaceted processes that go beyond individual economic considerations. Understanding what drives SSDI beneficiaries to return to work can help VR counselors provide individually tailored services to assist with a smooth transition off benefits and into employment.

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## **Conflict of interest**

None to report.

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